MILD/Moderate Substance Use Disorder

- ☑ Outpatient department (OPD) treatment:
  - Motivational intervention
  - Express empathy and listening, bringing out positives in life and that the drugs as stumbling blocks toward reaching the goals
  - Enhancing self-efficacy to resist drug use and rolling with resistance
  - Request to sign a contract of life.

Manage any comorbidity, e.g., specific learning disability (SLD) and depression. Adolescent may require substance-specific counseling, e.g., for marijuana. OPD treatment on an individual basis or in a group format to be delivered during a visit lasting 3 hours, once or twice a week.

Severe Substance Use Disorder

Pediatrician should refer such cases to multidisciplinary center/an adolescent-friendly psychiatrist for cognitive behavior therapy, motivational intervention, family therapy, and pharmacotherapy. These adolescents either require OPD treatment of 4–6 hours a day, 5 days a week while living at home, or may benefit from residential treatment—24-hour structured environment of a resource-intense high level of care.

There are two approaches for managing these adolescents.

Behavioral Approaches

- ☑ **Adolescent community reinforcement approach (A-CRA):** Achieve and maintain abstinence from drugs by replacing influences in their lives that had reinforced substance use.
- ☑ **Cognitive behavioral therapy (CBT):** A core element of CBT is teaching participants how to anticipate problems and helping them develop effective coping strategies. In CBT, adolescents explore the positive and negative consequences of using drugs. They learn to monitor their feelings and thoughts and recognize distorted thinking patterns and cues that trigger their substance abuse; identify and anticipate high-risk situations; and apply an array of self-control skills, including emotional regulation and anger management, practical problem solving, and substance refusal.
- ☑ **Contingency management (CM):** Provides adolescents an opportunity to earn low-cost incentives such as prizes or cash vouchers.
- ☑ **Motivational enhancement therapy (MET):** Motivational interviewing sessions in which a therapist helps the patient develop a desire to participate in treatment by providing nonconfrontational feedback and helps adolescents resolve their ambivalence about engaging in treatment and quitting their drug use.
- ☑ **Twelve-step facilitation therapy:** As in Alcoholics Anonymous (AA) or Narcotics Anonymous (NA) stress the participant's acceptance that life has become unmanageable, that abstinence from drug use is needed, and that willpower alone cannot overcome the problem.

Severe Substance Use Disorder

Family-based Approaches

- ☑ **Brief strategic family therapy (BSFT):** Over the course of 12–16 sessions, the BSFT counselor establishes a relationship with each family member, observes how the members behave with one another, and assists the family in changing negative interaction patterns.
- ☑ **Family behavior therapy (FBT):** The adolescent and at least one parent participate in treatment planning and choose specific interventions from a menu of evidence-based treatment options. Therapists encourage family members to use behavioral strategies taught in sessions and apply their new skills to improve the home environment.
- ☑ **Multidimensional family therapy (MDFT):** A comprehensive family- and community-based treatment useful for substance-abusing adolescents with behavior problems such as conduct disorder and delinquency.
- ☑ **Functional family therapy (FFT):** Combines a family systems view of family functioning (which asserts that unhealthy family interactions underlie problem behaviors) with behavioral techniques to improve communication, problem-solving, conflict resolution, and parenting skills.
- ☑ **Multisystemic therapy (MST):** MST is a comprehensive and intensive family- and community-based treatment for severe cases.

Recovery Support Services

These include mutual help groups, e.g., AA and NA, peer recovery support services, assertive continuing care (ACC), recovery high schools, etc. Support systems are mandatory for recovery to be sustained and free of relapse.

Further Reading

- ☑ Indian academy of pediatrics. Mission Kishore Uday Manual 2018-19. Mumbai, India: Indian academy of pediatrics; 2019.
- ☑ Kliegman RM, St. Geme J. Adolescent substance abuse. In: Kliegman RM, St. Geme J (Eds). Nelson Textbook of Pediatrics, 21st edition. Philadelphia, PA: Saunders Elsevier; 2019.
- ☑ National Institute on Drug Abuse. (2011). Preventing Drug Use among Children and Adolescents (In Brief). [online] Available from: https://nida.nih.gov/publications/preventing-drug-use-among-children-adolescents/prevention-principles. [Last accessed August, 2022].
- ☑ Sathanantham S, Dayasiri K, Thadchanamoorthy V. Approach to the adolescent with substance use in the acute setting. Cureus. 2021;13(7):e16309.

Indian Academy of Pediatrics (IAP)

STANDARD TREATMENT GUIDELINES 2022

Rabies Prophylaxis in Children

Lead Author

Anurag Agarwal

Co-Authors

Shivaprakash Sosale, Ashwath Narayan

Under the Auspices of the IAP Action Plan 2022

Remesh Kumar R
IAP President 2022

Upendra Kinjawadekar
IAP President-Elect 2022

Piyush Gupta
IAP President 2021

Vineet Saxena
IAP HSG 2022–2023

© Indian Academy of Pediatrics

IAP Standard Treatment Guidelines Committee

Chairperson

Remesh Kumar R

IAP Coordinator

Vineet Saxena

National Coordinators

SS Kamath, Vinod H Ratageri

Member Secretaries

Krishna Mohan R, Vishnu Mohan PT

Members

Santanu Deb, Surender Singh Bisht, Prashant Kariya, Narmada Ashok, Pawan Kalyan